CURELINK

DEPI Graduation Project

Project Documentation

Parts 1–3

Project Planning & Management · Literature Review · Requirements Gathering

TRACK	Software Development — Cross Platform Mobile App Developer
TEAM LEADER	Mohamed Yasser
TEAM MEMBERS	Ahmed Abdelbar · Hana Elganagy · Amira Galal · Abdelrahman Bassam · Veronica Kamil
PROJECT	Medication management and caregiver support mobile application

October 2026

Prepared as one submission PDF for the first three DEPI documentation parts.

Submission scope: Parts 1–3 only • Design/architecture artifacts are reserved for the later DEPI stage.

Document Control & Submission Scope

Item	Details
Project	CureLink
Document	First three required documentation parts in one PDF
Prepared for	DEPI Graduation Project
Official workstreams	Project Planning & Management; Literature Review; Requirements Gathering; System Analysis & Design; Implementation; Final Presentation, Testing & Reports
Relevant first-three period	Planning & Management: 2 Oct–16 Oct 2026; Literature Review: 9 Oct–16 Oct 2026; Requirements Gathering: 9 Oct–16 Oct 2026
Repository	github.com/Mohamedyasser9308/CureLink

What this PDF contains

- Part 1: Project Planning & Management — proposal, project plan, task assignment and roles, risk assessment/mitigation, and KPIs.
- Part 2: Literature Review — research context, similar-solution comparison, lecturer feedback/evaluation, suggested improvements, and clearly labeled evaluation criteria.
- Part 3: Requirements Gathering — stakeholder analysis, user stories/use cases, functional requirements, and non-functional requirements.

Important scope decision	The supplied DEPI guideline places **Wireframes & Mockups** and **UI/UX Guidelines** under Part 4: System Analysis & Design. Those design artifacts are therefore reserved for the later documentation stage.

Evidence discipline	Project-specific facts are stated as supplied by the team. Planned targets are labeled as proposed; missing lecturer comments and official grading percentages are not invented.

DEPI scope check

This submission deliberately contains Project Planning & Management, Literature Review, and Requirements Gathering. Use-case diagrams, architecture, database/DFD/sequence/activity/state/class diagrams, wireframes/mockups, UI/UX guidelines, deployment/component diagrams, and similar design artifacts are reserved for the later DEPI stage.

1. Project Planning & Management

1.1 Project Proposal

Overview. CureLink is a cross-platform mobile application concept focused on medication management and caregiver support. The planned product helps patients organize medication schedules, record dose status, and review medication information while allowing an authorized caregiver to access permitted information.

Problem. Medication routines can become difficult to manage when users have multiple medicines, changing schedules, or need support from a family member or caregiver. The project addresses organization, reminders, dose-status recording, and controlled caregiver access without presenting the app as a replacement for professional medical advice.

1.2 Project Objectives

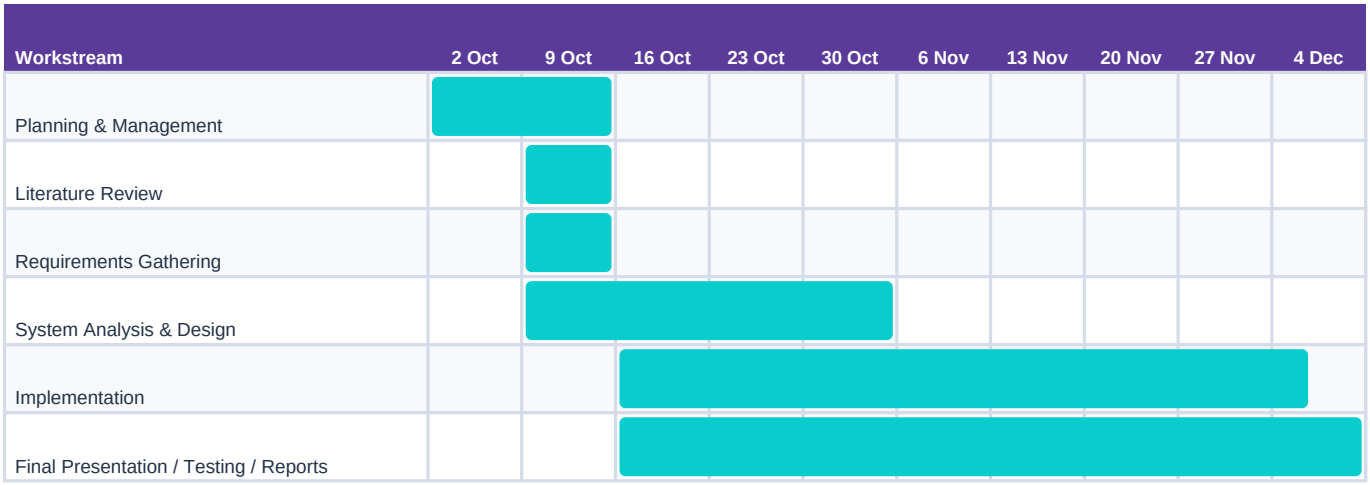
- Provide medication records and schedules with add, view, edit, and delete operations.
- Support reminders and dose-status recording such as Taken and Missed, with history.
- Enable controlled patient–caregiver linking with explicit permission levels.
- Provide role- and age-aware experiences for Child, Teen/Adult, and Older Adult users.
- Support account creation, sign-in, profile/session state, and role-based navigation.
- Apply usability, accessibility, localization, light/dark theme, and consistent shared UI principles.

1.3 Project Scope

In scope	Out of scope / not promised
Account registration and login; saved profile and role information.	Diagnosis, prescribing, dosage recommendations, or changing a clinician's treatment plan.
Medication records, schedules, details, dose status, and history.	Guaranteeing clinical outcomes or perfect medication adherence.
Patient–caregiver linking, approval, and explicit permissions.	Unrestricted caregiver access without patient approval.
Reminder and missed-dose notification flow where supported and configured.	Guaranteed emergency response or guaranteed SOS dispatch.
Age-group-aware UI, localization foundation, themes, and loading/empty/error states.	Clinical certification or production-grade reliability without evidence.

1. Project Plan, Gantt & Deliverables

1.4 Official Gantt Timeline



Workstream	Official period	Primary output
Project Planning & Management	2 Oct – 16 Oct 2026	Scope, plan, roles, risks, KPIs
Literature Review	9 Oct – 16 Oct 2026	Research context, comparison, evaluation/improvements
Requirements Gathering	9 Oct – 16 Oct 2026	Stakeholders, user stories/use cases, FR/NFR
System Analysis & Design	9 Oct – 6 Nov 2026	Analysis and design artifacts
Implementation (Source Code & Execution)	16 Oct – 30 Nov 2026	Integrated application and executable build
Final Presentation & Testing & Reports	16 Oct – 4 Dec 2026	Testing evidence, reports, presentation

Milestones & Deliverables

- First three documentation baseline: scope, research, stakeholder analysis, user stories/use cases, functional and non-functional requirements.
- Later phases: System Analysis & Design, Implementation, Testing/Final Presentation & Reports.
- All documents are to be uploaded to the team repository as required by the DEPI guideline.

Resource Allocation

Resource	Planned use in CureLink
Flutter / Dart	Cross-platform mobile application development.
Firebase Authentication	Account registration, sign-in, and authentication state.
Cloud Firestore	Planned persistence for profiles, medication records, schedules, dose logs, and caregiver links.
FCM / Cloud Functions	Planned notification and scheduled/missed-dose processing; implementation must be verified.
GitHub / Trello	Source control, branching/integration, task assignment, checklists, and coordination.

1. Task Assignment & Roles

1.5 Team Responsibilities

Team member	Week 1 assignment	Week 2 assignment	Role / coordination
Mohamed Yasser	Profile creation; profile image; age group; unique ID generation.	Empty/error states; age-group UI; dark mode; loading; review and merge.	Team leader; organizes tasks and merges branches.
Ahmed Abdelbar	Project structure; theme; light/dark foundation; localization foundation; shared widgets; app colors; Firebase initialization.	Patient home UI; medication list; next dose; home states/navigation; localization.	Foundation and patient-home work.
Hana Elganagy	Splash; three onboarding screens; Next/Skip/Get Started.	Medication model/repository; Add Medicine UI; validation; medicine image; save medicine; localization.	Onboarding and medication creation flow.
Abdelrahman Bassam	Login UI; email/password validation; Firebase Auth; login states; loading/error handling.	Edit medicine; validation; edit image; update medicine; testing; localization.	Authentication and edit-medication flow.
Amira Galal	Sign Up UI; Firebase registration; validation.	Medicine Details UI; load medicine; delete medicine; image handling; testing; localization.	Authentication registration and medicine-details/delete work.
Veronica Kamil	Role selection; save role; role-based navigation.	Session management; account state; logout; session testing; navigation integration.	Role and session flows.

Team Collaboration Approach

- Tasks are divided through Trello weekly cards and checklist items.
- Team members work on separate branches and integrate through the shared GitHub repository.
- The team leader reviews and merges contributions; shared routes, models, interfaces, and Firebase structures should be agreed before dependent merges.
- Parallel development requires clear ownership and integration testing before final delivery.

Execution & collaboration status

The supplied Trello export shows the listed Week 1 and Week 2 task checklists as completed (100%).

Team members work on separate branches, push changes to the shared GitHub repository, and the team leader reviews and merges contributions.

1. Risk Assessment & KPIs

1.6 Risk Assessment & Mitigation Plan

Risk	Likelihood / impact	Mitigation
Firebase configuration or integration errors	Medium / High	Document setup; test authentication/database access early; use a shared configuration checklist.
Notification permissions, delivery, or background limits	Medium / High	Test supported devices/OS versions; handle denied permissions; document platform limitations.
Incorrect role/permission exposure	Medium / High	Enforce access at the data layer; test read-only vs management permissions and approval states.
Merge conflicts / incompatible interfaces	High / Medium	Use feature branches, small changes, consistent naming, agreed models/routes, and review before merge.
Schedule pressure near deadlines	Medium / High	Prioritize critical flows; track checklists; reserve integration and regression-test time.
Localization/theme inconsistency	Medium / Medium	Use shared resources; review light/dark and age-group experiences together.
Medication data inconsistency	Medium / High	Validate inputs; handle loading/error states; verify CRUD and dose-state transitions.
Sensitive information mishandling	Medium / High	Collect only required data; apply authentication/least privilege; avoid unnecessary sensitive logging.
Accessibility issues	Medium / Medium	Review contrast, text size, touch targets, language simplicity, and age-aware layouts.

1.7 Key Performance Indicators

These are **proposed acceptance targets**, not measured results. Actual values should be recorded during testing and mapped to the official course rubric if one is provided.

KPI	Proposed target / measurement
Critical-flow test pass rate	100% of defined critical-path tests pass before the final demo; report passed / total.
Medication CRUD correctness	All planned add/view/edit/delete test cases pass with correct persisted data.
Authorization correctness	All role and caregiver-permission tests pass; unauthorized access attempts are denied.
Validation & state coverage	Required invalid inputs are rejected; critical loading/empty/error/success states are covered.
Localization / theme consistency	No critical missing localization keys; critical screens reviewed in supported display modes.
Repository integration	All intended release changes are merged and the final branch builds successfully in the agreed environment.
Notification-flow reliability	Defined reminder/missed-dose scenarios are tested and outcomes recorded; no unverified 100% delivery claim.

2. Literature Review

2.1 Review Purpose & Approach

This review examines the medication-management problem, evidence on mobile medication-adherence interventions, and publicly described features of comparable applications. It is a focused project review rather than a full systematic review. Research sources are used for evidence; official product pages are used to describe publicly advertised features rather than to claim independent clinical effectiveness.

2.2 Background: Medication Adherence & Digital Support

- The World Health Organization describes adherence to long-term therapies as a multifactorial issue that extends beyond simply sending reminders.
- A 2020 systematic review/meta-analysis of randomized controlled trials reported higher self-reported medication adherence among app users than comparison groups, while noting the need for further evaluation.
- A 2025 systematic review/meta-analysis covering 26 studies and 5,174 participants reported a positive association between mobile-health interventions and medication adherence; interactive strategies, reminders, and data-sharing were among the features examined.
- These findings support exploring reminders, dose tracking, and shared visibility in CureLink, but they do not establish clinical effectiveness of CureLink itself.

2.3 Research-to-Project Implications

Evidence / observation	Implication for CureLink
Medication adherence is influenced by multiple factors.	The app should support organization and visibility rather than present itself as a clinical treatment.
Reminder and interactive strategies are relevant in mobile interventions.	Reminder and dose-status flows should be clear and testable.
Data-sharing can support caregiver involvement.	Sharing should be consent-based with explicit permission levels.
Evidence varies by population and outcome measure.	The project should evaluate usability and functional behavior instead of claiming medical outcomes.

2. Similar Solutions & Comparison

2.4 Comparable Solutions

Solution	Publicly described strengths	Considerations	Relevance to CureLink
Medisafe	Medication reminders and dose tracking; caregiver-oriented options including missed-dose alerts and dependent profiles.	Caregiver models and permissions may differ; features are summarized from public help documentation, not independently tested here.	Supports caregiver support and explicit permission distinctions.
MyTherapy	Medication reminders, intake records, and a broader health diary are described on its official product page.	Its scope is broader than CureLink's role- and age-group-centered focus; no clinical outcome comparison is made here.	Supports dose history and consolidated medication information; CureLink adds explicit caregiver roles/permissions.
Research evidence	Systematic reviews indicate potential benefits from mobile medication-adherence interventions.	Studies differ in population, intervention, and outcome; app usage does not prove long-term clinical benefit.	Supports testing reminder, dose-status, and sharing flows rather than promising outcomes.

2.5 Design / Product Implications for CureLink

- **Consent-based caregiver access:** approval and permission levels should be explicit.
- **Actionable medication history:** Taken and Missed states should be distinguishable and traceable.
- **Age-aware usability:** child and older-adult experiences should be reviewed for readability and simplicity.
- **Transparent reminders:** permissions and platform delivery limitations should be communicated.
- **Privacy and safety:** CureLink should remain a medication-organization/support tool and avoid implying diagnosis or prescribing.

Source boundary	The comparison uses WHO and peer-reviewed research plus official Medisafe/MyTherapy materials. Product descriptions are not treated as independent product tests, and research findings are not presented as proof that CureLink will improve adherence.

2. Feedback, Improvements & Evaluation

2.6 Lecturer Feedback & Evaluation

Item	Documented status
Initial concept discussion	The team reported that the lecturer responded positively to the CureLink idea.
Detailed comments	No exact written comments or detailed lecturer evaluation were supplied. No specific recommendation is attributed to the lecturer.
Action	Any later lecturer feedback should be recorded accurately and used to update requirements/design decisions.

2.7 Suggested Improvements

Priority	Improvement	Reason
High	Validate caregiver permissions and patient approval end-to-end.	Medication data is sensitive; access must match patient choice and role.
High	Test reminder, missed-dose, and caregiver-alert scenarios on supported devices.	Reminder behavior and delivery depend on implementation and platform constraints.
High	Use explicit dose states and clear history.	Prevents ambiguity between a user-marked dose and a system-generated missed state.
High	Provide clear loading, empty, error, and retry states.	Reduces confusion during network or data-operation failures.
Medium	Review contrast, text size, touch targets, and language simplicity.	Supports the age-aware and accessibility goals of the project.
Medium	Run a small usability walkthrough with representative users/proxies, with consent and no real medical data.	Can identify confusing flows before final presentation; it is formative feedback, not clinical validation.
Medium	Document supported OS/device assumptions and notification limitations.	Improves reproducibility and realistic expectations.

2.8 Final Grading / Evaluation Criteria

Important

The supplied DEPI guideline requests a grading breakdown but does not provide official percentages. The dimensions below are therefore **proposed evaluation areas**, not official DEPI marks.

Evaluation area	Evidence to present
Functional completeness	Demonstrate the agreed critical user journeys.
Correctness & validation	Show validation, persistence, dose-state behavior, and error cases.
Security & privacy	Demonstrate authentication, consent, caregiver permissions, and restricted access.
UI quality & accessibility	Show consistency with the approved design and age-aware presentation in the later design stage.
Testing quality	Present test cases, outcomes, defects, and fixes; separate measured results from targets.
Code quality & collaboration	Show readable structure, branching/merge history, and an integrated build.
Documentation & presentation	Provide the required documents, references, coherent demonstration, and honest limitations.

3. Requirements Gathering

3.1 Stakeholder Analysis

Stakeholder	Needs / interests	Requirement implication
Patient — Child	Simple medication-focused interactions, clear reminders, age-appropriate feedback, adult support.	Patient-only profile; simple interactions; reward elements are motivational UI, not medical claims.
Patient — Teen/Adult	Manage medicines/schedules, record status, view history, choose caregiver linkage.	Patient mode is the default for Both; mode switching follows project rules.
Patient — Older Adult	Readable text, strong contrast, straightforward navigation, reminders, support.	Larger text, clear actions, forgiving error handling.
Caregiver	See linked patients and permitted information; receive configured alerts; manage medicines only when authorized.	Caregiver dashboard, empty state, and permission enforcement.
Team / developers	Clear ownership, agreed interfaces, testable acceptance criteria, predictable integration.	Trello ownership, Git branches, review/merge workflow, shared models/routes.
Lecturer / evaluator	Understand problem, scope, rationale, progress, testing, and limitations.	Traceable requirements, evidence-based claims, demo scenarios, documentation.
Technical dependencies	Authentication, persistence, notification delivery, controlled backend operations.	Configuration, security rules, permissions, failure handling, platform constraints must be tested.

3.2 User Types, Roles & Access Rules

Type	Rule
Child	Patient-only; medication-focused experience with simple interactions.
Teen/Adult — Patient	Manage own medication records, schedules, dose status, history, and caregiver access requests.
Teen/Adult — Caregiver	Caregiver dashboard for approved linked patients and permitted information.
Teen/Adult — Both	Supports patient and caregiver modes; defaults to Patient Mode and provides a mode switch.
Older Adult	Patient-only; larger text and higher contrast.
Caregiver link	Access Request → Patient Approval → Connected; permission is Read-only or Manage Medications.

3. User Stories & Acceptance Criteria

US-01 — Account registration
As a new user, I want to create an account so that I can access the app.
Acceptance: Valid required details create an account and continue to role/profile setup; invalid inputs show actionable validation.

US-02 — Login
As a registered user, I want to sign in so that I can reach my saved information.
Acceptance: Valid credentials load the saved account/profile and route correctly; invalid credentials show an appropriate error.

US-03 — Role and profile setup
As a user, I want to select my applicable role and age group so that the app shows the correct navigation.
Acceptance: The selected role/age group is saved; Child and Older Adult are patient-only; Both is only for Teen/Adult and defaults to Patient Mode.

US-04 — Add medication
As a patient or authorized caregiver, I want to add a medication so that its details and schedule can be tracked.
Acceptance: Required fields are validated; valid data is saved; success/failure is communicated; the record appears in the relevant list.

US-05 — View medication details
As an authorized user, I want to open a medication record so that I can review its details and schedule.
Acceptance: The correct record loads; loading, missing-record, and error states are handled; missing images do not break the screen.

US-06 — Edit medication
As a patient or caregiver with management permission, I want to update a medication so that the record remains current.
Acceptance: Changes are validated and persisted; read-only users cannot edit; updated details appear after save.

US-07 — Delete medication
As a patient or caregiver with management permission, I want to delete a medication so that obsolete records are removed from active management.
Acceptance: A clear confirmation opportunity is provided; only the intended record is deleted; failures are communicated and the list stays consistent.

3. User Stories & Use Cases — Continued

US-08 — Record dose status
As a patient, I want to mark a scheduled dose as taken or missed so that my medication history reflects my actions.
Acceptance: The selected dose/date-time is recorded correctly; repeated actions do not silently create conflicting status records.

US-09 — Receive reminder
As a patient, I want reminders for scheduled doses so that I can be prompted at the appropriate time.
Acceptance: When notifications are permitted and supported, reminders follow the configured schedule; permission/delivery limitations are handled transparently.

US-10 — Link a caregiver
As a patient, I want to request caregiver access and approve it so that I can share selected medication information safely.
Acceptance: A request is not connected until approved; the selected permission level is stored and applied.

US-11 — Caregiver overview
As an approved caregiver, I want to see my linked patients and permitted medication information so that I can support them.
Acceptance: Only approved links are shown; read-only blocks changes; Manage Medications permits only authorized operations.

US-12 — Missed-dose alert
As an authorized caregiver, I want an alert for a linked patient's missed dose when configured so that I can follow up.
Acceptance: The alert is associated with the correct patient/dose and respects link/notification settings; delivery is not claimed unless confirmed.

US-13 — Logout and session state
As a user, I want to log out so that my account is not left open to the next person.
Acceptance: Logout clears the active authenticated session and returns to the expected entry screen; reopening follows current session state.

US-14 — Accessible presentation
As a user, I want the interface to fit my age group and display preference so that key actions are understandable.
Acceptance: Applicable screens respect age-group design and light/dark theme; critical text remains readable and controls are accessible.

3.3 Use Cases

Use case	Primary actor	Preconditions	Main outcome
Register and configure profile	New user	App installed; user not signed in.	Account, role, and profile details saved; correct experience opened.
Sign in / restore session	Existing user	Account exists.	Authentication state checked; saved role determines navigation.
Manage medication	Patient / authorized caregiver	Signed in; permission allows management.	Medication created, viewed, updated, or deleted with validation/error handling.
Record dose status	Patient	Relevant medication schedule exists.	Dose status and history updated consistently.
Request / approve caregiver access	Patient + caregiver	Both accounts exist.	Request approved/rejected; only approved links grant permission.
Review patient information	Approved caregiver	Patient link exists.	Caregiver sees permitted patient/medication/status information.
Handle missed-dose workflow	System + authorized caregiver	Schedule/reminder logic configured.	Defined missed-dose rule is applied and caregiver is notified where configured.
Logout	Signed-in user	Valid session exists.	Session ends and protected screens are no longer accessible.

3. Functional Requirements

ID	Functional requirement
FR-01	Allow new users to register and existing users to sign in using Firebase Authentication.
FR-02	Validate required registration/login inputs and provide loading, success, and error feedback.
FR-03	Save/load user profile and selected role; route according to role and applicable age group.
FR-04	Support account constraints: Child and Older Adult are patient-only; Teen/Adult can be Patient, Caregiver, or Both.
FR-05	For a Both account, default to Patient Mode and provide switching between patient and caregiver modes.
FR-06	Allow authorized users to create medication records with validated details and optional image handling.
FR-07	Display medication details and handle loading, missing-record, and retrieval-error states.
FR-08	Allow authorized users to update medication details and persist valid changes.
FR-09	Allow authorized users to delete the intended medication record and keep the active list consistent.
FR-10	Support medication schedules and show relevant upcoming/next-dose information.
FR-11	Allow a patient to record a dose as Taken or Missed and maintain dose-status history.
FR-12	Request/handle notification permission and support dose reminders within platform capabilities.
FR-13	Support caregiver access requests, patient approval/rejection, and connection state.
FR-14	Enforce Read-only and Manage Medications permissions for each approved caregiver–patient link.
FR-15	Show linked patients in the caregiver experience and a clear empty state when none are connected.
FR-16	Where implemented/configured, process missed-dose events and notify authorized linked caregivers.
FR-17	Support logout and correctly handle current authentication/session state on app launch.
FR-18	Provide appropriate loading, empty, error, and success states for critical data operations.
FR-19	Support the agreed localization resources and light/dark theme.
FR-20	Provide age-group-aware presentation consistent with the approved design.

Implementation boundary	Requirements describe the agreed project scope. They are not claims that every item is already complete; completion must be verified during implementation and testing.

3. Non-Functional Requirements & Data Rules

ID	Attribute	Requirement / verification
NFR-01	Security & privacy	Protected data requires authentication and appropriate authorization; access rules are tested at the data layer.
NFR-02	Reliability	Critical medication operations show clear outcomes and recover gracefully from service/network errors.
NFR-03	Usability	Primary actions use clear labels, feedback, and predictable navigation.
NFR-04	Accessibility	Contrast, text size, touch targets, and layouts are reviewed for applicable age groups and display modes.
NFR-05	Performance	Common screens/lists should load without avoidable blocking; loading states appear for noticeable operations; exact targets follow baseline testing.
NFR-06	Maintainability	Shared widgets, theme values, localized strings, and clear feature boundaries reduce duplicated UI logic.
NFR-07	Compatibility	The app is tested on agreed Android/emulator and target devices; supported OS/device range is documented.
NFR-08	Data integrity	Medication and dose-status changes apply to the correct record and avoid inconsistent duplicate/stale UI state.
NFR-09	Localization	User-facing text uses localization resources; supported languages and fallback behavior are documented.
NFR-10	Notification transparency	The app does not imply delivery/receipt unless confirmed; permission denial and platform restrictions are handled.
NFR-11	Testability	Critical flows have positive, negative, permission, and error-path test cases with reproducible results.
NFR-12	Safety boundaries	The documented scope is medication organization/support; no diagnosis or prescribing recommendations are generated.

3.5 Data & Permission Rules

- Every medication record is associated with the appropriate patient/profile; a user must not accidentally view or modify another person’s medication data.
- Caregiver access follows Access Request → Patient Approval → Connected. Pending or rejected requests do not grant access.
- Read-only caregivers may view only permitted information and cannot add, edit, or delete medication records.
- Manage Medications caregivers may perform medication operations only for patients who approved that permission.
- A patient-confirmed Taken status should remain distinguishable from a system-generated Missed status.
- Security rules must be tested independently of the visual interface.

3.6 Open Decisions to Validate During Implementation

- Exact supported languages, minimum OS versions, and supported devices.
- Whether all planned notification/Cloud Function flows are implemented in the final build.
- Exact grace-period logic and criteria for marking a dose Missed.
- Whether any emergency SOS concept shown in later UI/design artifacts is part of the functional implementation; no emergency-service guarantee is assumed here.

References & Source Notes

[1] World Health Organization. (2003). *Adherence to Long-Term Therapies: Evidence for Action*.

[2] Armitage, L. C., Kassavou, A., & Sutton, S. (2020). *Do mobile device apps designed to support medication adherence demonstrate efficacy? A systematic review of randomised controlled trials, with meta-analysis.* BMJ Open, 10(1), e032045.

[3] *Effectiveness of Mobile Health Intervention in Medication Adherence: A Systematic Review and Meta-Analysis.* (2025). PubMed.

[4] Medisafe Help Center. *Caregiver Guide: How to Use Medisafe to Manage a Family Member's Medications.*

[5] MyTherapy. *Medication Reminder and Pill Tracker App.*

[6] U.S. Food and Drug Administration. *Policy for Device Software Functions and Mobile Medical Applications.*

[7] CureLink team. (2026). *CureLink Trello board export* — task assignments and checklists.

[8] CureLink team. (2026). *CureLink source repository.* GitHub.

Source-use note

The literature section summarizes the supplied research and official product/guidance sources rather than reproducing them. Project requirements, risks, KPIs, and acceptance criteria are project-specific synthesis based on the team's supplied context. Proposed evaluation criteria are explicitly labeled because no official DEPI grading percentages were provided in the supplied guideline.

Final document boundary	This PDF intentionally stops at **Parts 1–3**. The guideline places System Analysis & Design—including use-case diagrams, architecture, database/DFD/sequence/activity/state/class diagrams, wireframes/mockups, UI/UX guidelines, deployment/component diagrams, and related design artifacts—in the later documentation stage.